

DOPATAB M250 TABLET

VIDOP01-var

DESCRIPTION

Round, pale yellow film-coated tablet, bevel-edged, shallow convex faces.

COMPOSITION

Methyldopa equivalent to anhydrous Methyldopa 250 mg per tablet.

ACTIONS AND PHARMACOLOGY

The exact mechanism of antihypertensive action is unknown. It is thought to involve stimulation of central alpha-adrenergic receptors by a metabolite, alpha-methylnorepinephrine, thus inhibiting sympathetic outflow to the heart, kidneys and peripheral vasculature. Reduced peripheral resistance and plasma renin activity levels may also contribute to its effect.

PHARMACOKINETICS

Absorption:

Absorption of methyldopa from the gastrointestinal tract is variable but averages approximately 50%.

Protein Binding:

Methyldopa - Low (less than 20%);
Sulphate conjugate - Moderate

Biotransformation:

It is extensively metabolised and is converted to alpha-methylnorepinephrine in central adrenergic neurons; methyldopate hydrochloride is hydrolysed to methyldopa.

Sulphate conjugation occurs to a greater extent after oral than after intravenous administration.

Half-life:

Normal - Alpha: 1.7 hours;
Anuric - Alpha: 3.6 hours

Time to peak effect:

Single dose - 4 to 6 hours;
Multiple doses - 2 to 3 days

Duration of action:

Single dose - 12 to 24 hours;
Multiple doses - 24 to 48 hours

Excretion:

Renal; approximately 70% of absorbed drug is excreted in urine as methyldopa and its mono-O-sulfate metabolite.

Unabsorbed oral methyldopa is excreted unchanged in the feces.

INDICATION

For treatment of moderate to severe hypertension.

CONTRAINDICATION

Not recommended for patients with acute liver disease or pheochromocytoma.

PRECAUTIONS

- Evidence of hemolytic anaemia is an indication for discontinuing the drug.
- Positive Coombs' tests may indicate an incompatible cross-match when transfusion is required and an indirect Coombs' test should be carried out.
- Safety for use in pregnancy and lactation has not been established.
- Caution if any kind of surgery or dialysis is required.

PREGNANCY AND LACTATION

Safety for use in pregnancy and lactation has not been established.

MAIN SIDE/ADVERSE EFFECTS

Drowsiness, edema, dry mouth and headache.

DRUG INTERACTIONS

- Concurrent use with the following drugs is not recommended:
Monoamine oxidase (MAO) inhibitors, levodopa and methotrimeprazine.
- Concurrent use with the following drugs requires careful monitoring:
Alcohol, antihypertensives, general anaesthetics and norepinephrine.

OVERDOSAGE

Clinical features:

Dizziness, drowsiness, hypotension, tachycardia or bradycardia, hypothermia, extra-pyramidal signs, coma.

Treat overdosage by emesis or gastric lavage, if appropriate; and IV 1-2 mg benztropine if extrapyramidal signs are present.

DOSAGE AND ADMINISTRATION

Adults:

Initial - Oral, 250 mg two or three times a day for two days, the dosage then being adjusted as directed.

Maintenance - Oral, 500 mg to 2 g a day, divided into two to four doses, not exceeding 3 g daily.

Children:

Oral, initially 10 mg per kg of body weight daily, divided into two to four doses, the dosage then being adjusted as directed, not exceeding 65 mg per kg of body weight or 3 g daily, whichever is less.

Storage: Store below 30°C. Protect from light.

Presentation/Packing: Blister of 10 x 10's

Product Registration Holder: HOVID Bhd.

121, Jalan Tunku Abdul Rahman (Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

Manufactured by: HOVID Bhd.

Lot 56442, 7 1/2 Miles, Jalan Ipoh/Chemor,
31200 Chemor, Perak, Malaysia.

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